

HOW TO OPEN A NON-MEDICAL HOME CARE AGENCY in South Carolina

A DPH-licensed In-Home Care Provider — private pay & Medicaid (CLTC waivers).

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The South Carolina model — read this first

In South Carolina, a business that sends caregivers into people's homes for personal care, companionship, or homemaker help must be licensed by the South Carolina Department of Public Health (DPH) as an In-Home Care Provider (IHCP), under SC Code Title 44, Chapter 70 and Regulation 61-122. (Health-facility licensing moved to the new DPH when DHEC split in 2024.) The IHCP license is relatively light-touch on staffing — no minimum training hours, no RN-supervision mandate — but it IS a real license with a \$1,000 fee, and unlike Florida, South Carolina does NOT broadly exempt companion/sitter businesses. This guide walks the whole path — licensing, then getting paid — broken down so a first-timer can follow it.

You need the license — companion/sitter isn't broadly exempt. South Carolina's In-Home Care Provider law requires a license before you operate or even advertise as an in-home care provider. The only exemptions are narrow: house-cleaning-only services, a caregiver an individual/family hires directly, and unpaid religious 501(c)(3) care. A company offering companion/sitter/homemaker plus any personal care must be licensed. (If your service is truly pure companionship with zero help with daily living, confirm your status with DPH before assuming you're exempt.)

Accreditation — optional here. South Carolina does NOT require accreditation for an IHCP license. Accreditation (ACHC — headquartered in nearby Cary, NC — CHAP, or The Joint Commission) is often preferred by VA and managed-care contracts and helps win referral partners. Every PolicyReady product is built to those standards behind the scenes; treat accreditation as a goal for year two (Part 9).

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened an agency — where we name a thing, we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with DPH, SC Medicaid, and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

1	Decide this is your business	A licensed in-home care agency — recurring, hourly revenue (Part 1)
2	Form the business & insure	Entity, EIN, NPI, and the right insurance & bonding (Part 3)
3	Get the DPH license	The IHCP application, the \$1,000 fee (Part 4)
4	Set up your administrator	The administrator and your operating policies (Parts 4–5)
5	Recruit & screen caregivers	Training, SLED check + drug test, TB (Part 5)
6	Set up EVV & operations	AuthentiCare EVV, policies, systems (Part 6)
7	Serve private-pay clients	Start earning while Medicaid

		enrollment proceeds (Part 8)
8	Enroll for Medicaid	SCDHHS enrollment + the CLTC waiver personal care (Part 8)
9	Get your first clients	Discharge planners, the 10 AAAs, the Aging Helpline (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, in one place (Part 11)

Rule of thumb. Get your entity and policies in order, apply to DPH, and launch on private pay. South Carolina's staffing rules are light, so your differentiator is quality — train and screen well. Medicaid waiver personal care stays fee-for-service (not managed care), so enroll with SCDHHS/CLTC for that volume.

PART 1

What This Business Is

In short: A DPH-licensed In-Home Care Provider — caregivers who help clients with daily living in their own homes. Personal care, companion, and homemaker work under one license.

South Carolina licenses In-Home Care Providers to deliver, for compensation, in-home help with the activities of daily living — walking, getting in and out of bed, bathing, dressing, feeding, using the toilet, preparing special diets, and supervising self-administered medication — plus homemaker and companion services. It's the non-medical home-care license; skilled/medical home health is a separate, heavier license (Home Health Agency under Reg 61-77).

In-home care vs. skilled home health

	In-Home Care Provider (this guide)	Home Health Agency (skilled)
What it is	Personal care, companion, homemaker	Nursing, therapy, medical care
Regulation	Reg 61-122 (DPH)	Reg 61-77 (DPH)
Who delivers it	Trained caregivers	RNs, LPNs, therapists
Medications	Supervise self-administration only	Nurses administer/manage medications

Key terms. DPH = SC Department of Public Health (your licensor, since the 2024 DHEC split). IHCP = In-Home Care Provider (your license). Reg 61-122 = the licensing standards. SCDHHS = SC Dept. of Health & Human Services (Medicaid). CLTC = Community Long Term Care / Division of Long Term Living (runs the waivers). Community Choices = the main HCBS waiver covering personal/attendant care. SLED = SC Law Enforcement Division (background checks). EVV = Electronic Visit Verification (AuthentiCare). NPI = National Provider Identifier.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients “quicker and sicker,” which sends more families looking for help at home right after a hospital stay. South Carolina is one of the fastest-aging states in the Southeast — its 65-and-over population is growing quickly in the coastal and Upstate retirement corridors while a large rural population needs care brought to the home — and it's still a relatively open, low-barrier market to enter. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) undercapitalization — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) weak caregiver recruiting and retention — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) sloppy compliance — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

Who Regulates You (and What Each One Does)

In short: DPH licenses and inspects you, SCDHHS/CLTC pays you for Medicaid waiver personal care (fee-for-service), SLED screens your staff, and AuthentiCare verifies Medicaid visits.

SC DPH — Healthcare Facility Licensing (your licensor)

The South Carolina Department of Public Health licenses In-Home Care Providers under Reg 61-122, reviews your application and policies, and conducts inspections/investigations. Healthcare Facility Licensing main line (803) 545-4215; dph.sc.gov (In-Home Care Providers) — confirm the current IHCP intake contact.

SCDHHS / CLTC and Medicaid

SC Medicaid (Healthy Connections), through its Division of Long Term Living (formerly CLTC), runs the HCBS waivers that pay for personal and attendant care. Importantly, effective January 1, 2026, waiver and nursing-facility services are authorized and paid through fee-for-service (SCDHHS/CLTC) — NOT the managed-care plans. Provider Service Center: (888) 289-0709.

SLED (background checks)

The South Carolina Law Enforcement Division runs the criminal-background checks your caregivers must clear before employment (plus a drug test), per Reg 61-122.

EVV — AuthentiCare

For Medicaid personal-care visits, South Carolina uses AuthentiCare (a single state-mandated EVV system, not an open aggregator). Visits are documented in EVV at check-out; a visit must be verified to be paid. Private-pay work isn't subject to EVV.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In South Carolina you form the LLC by filing Articles of Organization with the Secretary of State — online at sos.sc.gov (the businessfilings.sc.gov portal) — for a \$110 filing fee. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

1. **File your Articles of Organization.** Online at sos.sc.gov / businessfilings.sc.gov — the filing fee is \$110.
2. **Name a registered agent.** A person or service with a physical South Carolina address who receives legal mail — this can be you.
3. **Adopt an Operating Agreement.** The internal document that sets who owns what, who manages the company, and how money is handled.
4. **No LLC annual report — but get a LOCAL business license.** South Carolina LLCs don't file an annual report (only corporations do). There's no statewide general business license, so check your county and city for the local one you need.

No newspaper publication in South Carolina. *Forming the LLC is a straightforward online filing — no publication requirement like New York's. Get your EIN next (free from the IRS), and if you'll bill Medicaid or the VA, a Type 2 NPI (free from NPPES).*

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

1. **Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
2. **Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
3. **Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
4. **Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll with SC Medicaid and bill CLTC waiver personal care. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

1. **Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
2. **Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a home-care / personal-care agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of South Carolina branches — Bank of America, Wells Fargo, Truist, South State Bank (SC-based), or a local credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A home-care agency sends caregivers into clients' homes, so you need coverage built for that specific risk — not an off-the-shelf general business policy:

Coverage	What it protects against	Must-have?
General & Professional Liability	A slip-and-fall or property damage in a client's home, plus claims about the CARE (a caregiver's error or neglect)	Yes — foundational
Abuse & Molestation	Allegations of abuse by a caregiver — general liability EXCLUDES this	Yes — high exposure
Employee Dishonesty / Surety Bond	Theft from a client's home — families ask if caregivers are “bonded”	Yes — clients & payers expect it
Non-Owned & Hired Auto	Your caregivers driving their own cars to clients or with clients	Yes — critical for home care
Workers' Compensation	Caregiver injuries on the job	Required in SC at 4+ employees

South Carolina workers' comp kicks in at four employees. South Carolina requires workers' compensation once you regularly employ four or more people (part-timers count). A home-care agency crosses that line fast, so set it up with your payroll provider or broker before you do — don't wait for the fourth hire.

Real places to get it

Buy through an independent agent or broker who writes home-care programs — carriers and programs active in this niche include Philadelphia Insurance (PHLY), CNA, The Hartford, Markel, and home-care specialty programs from Caitlin Morgan and NIP Group / HomeCareUSA. To find a South Carolina agent, use [trustedchoice.com](https://www.trustedchoice.com) (filter for home care), or ask a franchise or peer. Confirm the carrier writes non-medical home care in South Carolina before you rely on a quote.

What to ask the insurance agent

“Do you write South Carolina In-Home Care Providers — which carriers, and what's the minimum premium?”

“Is abuse & molestation included or a sub-limit, and do you include non-owned & hired auto (my caregivers drive their own cars)?”

“Can you place an employee-dishonesty / surety bond so I can tell clients my caregivers are bonded?”

“What coverage limits do SC Medicaid, the VA, or long-term-care insurers require?”

“When do I need South Carolina workers' comp given my caregiver headcount?”

PART 4

Get Your DPH License — Step by Step

In short: Assemble your policies and application, pay the \$1,000 fee, and submit to DPH Healthcare Facility Licensing. South Carolina's staffing rules are light, so a clean, professional application moves smoothly.

4.1 Before you apply

- ☐ Entity formed; EIN and NPI obtained; local business license
- ☐ Insurance & bond bound
- ☐ A designated administrator identified (Part 5)
- ☐ Your policies & procedures written (your P&P Manual) — client care, staff training, background checks, TB screening, complaints, incidents

4.2 The application

1. **Get the IHCP application.** From the DPH Applications for Licensure / Forms hub (dph.sc.gov).
2. **Submit with your policies.** Provide ownership, services, and your operating policies to Reg 61-122.
3. **Pay the fee.** The initial IHCP license fee is \$1,000 (annual renewal \$800).
4. **Complete DPH review.** DPH reviews the application; inspections/investigations are used for enforcement (Reg 61-122 doesn't mandate a specific pre-opening survey — confirm current practice with DPH).

4.3 Timeline

DPH doesn't publish a fixed turnaround, so submit a complete application and start your Medicaid enrollment and caregiver hiring in parallel. Launch on private pay as soon as you're licensed.

PART 5

Your Administrator & Caregivers — by Position

In short: South Carolina keeps staffing rules light — no minimum training hours, no RN-supervision mandate — but you still screen hard and train to a real standard. Quality is your differentiator here.

5.1 Your administrator

Reg 61-122 requires a designated administrator — the person with authority and responsibility to manage the provider — but does not set specific education or credential requirements. This can be you. Define the role in a written job description and be ready to show DPH your management structure.

5.2 Your caregivers — training & competency

- **Training (Reg 61-122 §300.D):** caregivers must receive or obtain training covering basic first aid, medication assistance (if applicable), condition-specific care (e.g., dementia), confidentiality, documentation, ethics/interpersonal skills, proper lifting/transfer techniques (if applicable), and infection control — signed and dated by trainer and trainee. There's no minimum hour count or state exam, so build a solid orientation and skills check-off yourself.
- **TB screening (§400.A):** every staff member and caregiver with client contact needs a health assessment within 12 months before initial client contact, including TB screening per CDC/DPH guidance.
- **No nurse-aide registry / no RN supervision:** IHCP aides aren't certified nurse aides, and Reg 61-122 doesn't require RN supervision of personal-care aides — a genuinely light-touch standard.

5.3 Background checks — the system you must run

1. **Run the SLED criminal check + drug test.** Before employment, each caregiver undergoes a SC Law Enforcement Division criminal-background check and a drug test (Reg 61-122 §300.A).
2. **Honor the disqualifiers.** No one with convictions (or no-contest pleas) related to theft, abuse, neglect, or exploitation may be employed.
3. **Confirm the fingerprint question with DPH.** Whether DPH requires fingerprint/FBI checks (vs. SLED-only) for IHCP aides is ambiguous in the rule — confirm current practice with DPH before you rely on SLED-only.
4. **Document everything.** Keep dated results in each personnel file.

5.4 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

5.5 Building your caregiver screening & training pipeline (the South Carolina playbook)

Here is South Carolina's defining operational reality: the state sets NO minimum training hours, NO certification exam, and NO nurse-aide registry for IHCP caregivers. That is both the opportunity and the risk. In a state where

almost anyone can call themselves a caregiver, the agencies that win hospital and insurer referrals are the ones that screen hardest and train to a real, documented standard — because the state won't set that bar for you, so it becomes your differentiator. Here is how successful South Carolina owners build that pipeline:

- 1. Run every hire through SLED plus a drug test before day one.** Order the SC Law Enforcement Division criminal-background check and a drug screen before employment (Reg 61-122 §300.A), and confirm with DPH whether a fingerprint/FBI check is also expected. No one with a theft, abuse, neglect, or exploitation history is eligible — this is a hard stop, not a judgment call.
- 2. Deliver the §300.D training and a hands-on skills check-off.** Cover basic first aid, medication assistance, condition-specific care (e.g., dementia), confidentiality, documentation, ethics and interpersonal skills, safe lifting and transfers, and infection control — each topic signed and dated by trainer and trainee. Add a skills check-off in which a supervisor watches the caregiver perform key tasks before they ever work a shift alone.
- 3. Complete the health assessment and TB screening within the window.** Every caregiver with client contact needs a health assessment including TB screening within the 12 months before initial client contact (§400.A) — schedule it during onboarding so it never blocks a start of care.
- 4. Recruit continuously and onboard fast.** Post on Indeed and local Facebook groups, run a caregiver-referral bonus, and interview the same week — the best caregivers take the first agency that calls them back and puts them on a schedule.

Your training program IS your sales pitch. Because South Carolina won't certify your caregivers for you, keep a written orientation curriculum and signed, dated training records in every personnel file. When a hospital discharge planner or a long-term-care insurer asks why they should trust your caregivers over a cheaper competitor, a real, standards-based training program is the answer — and it's exactly what DPH looks for when it inspects.

PART 6

EVV, Policies & Getting Operational

In short: Write your policies, set up AuthentiCare EVV for Medicaid visits, and get your scheduling/billing systems in place. EVV is the gate to Medicaid payment.

6.1 Your policies & procedures

Build a P&P Manual to Reg 61-122: client rights and confidentiality, intake and the service plan, caregiver training and competency, background checks and drug testing, TB screening, complaint handling, incident and abuse reporting, and emergency preparedness. This is your operating backbone and what DPH reviews.

6.2 Set up EVV (before your first Medicaid visit)

South Carolina Medicaid uses AuthentiCare (Fiserv) as its single EVV system for personal care. Onboard, set up your caregivers/clients/authorizations, capture every visit at check-out (supervisory visits are also recorded in EVV), and bill against verified visits. Private-pay work isn't subject to EVV.

6.3 Systems

Choose a home-care platform for scheduling, records, invoicing, and payroll that can work with AuthentiCare for your Medicaid line.

6.4 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

The Client's Journey — Intake to Discharge

In short: *A client is referred (privately or via CLTC case management), you assess and set up the plan, a caregiver delivers the hours (with EVV for Medicaid), you supervise, and you review and adjust.*

Stage 1 — Referral & inquiry

A referral comes from a family, a hospital discharge planner, a home-health/hospice partner, or a CLTC case manager (Part 10). Take the basics — who, what tasks, how many hours, where, when, and who pays (private, Medicaid/CLTC, VA, or LTC insurance).

Stage 2 — Assessment & the service plan

Do an in-home assessment and build a service plan: the tasks, schedule, and hours. For CLTC waiver clients, the case manager sets the authorized service and hours; your plan aligns to it.

Stage 3 — Agreement & start of care

The client (or representative) signs a service agreement (services, schedule, rate/payer, rights, termination). Match a screened, trained caregiver and start with a supervised introduction.

Stage 4 — Delivering the hours (with EVV for Medicaid)

The caregiver delivers care; Medicaid visits are captured in AuthentiCare. Keep visit notes and protect the client-caregiver match — consistency drives satisfaction and retention.

Stage 5 — Supervision & quality

Supervise per your policy; monitor performance and changing needs; run your quality program; and report any suspected abuse, neglect, or exploitation.

Stage 6 — Changes & reassessment

As needs change, update the service plan; for CLTC clients, coordinate reassessments and new authorizations with the case manager. Keep authorizations current so delivered hours get paid.

Stage 7 — Discharge & transition

When care ends, follow your discharge policy, give notice, coordinate with the family and any payer, and document. A good discharge turns families into referral sources.

PART 8

How You Get Paid — Private Pay, Medicaid (CLTC), VA & LTC Insurance

In short: Start on private pay and long-term-care insurance the day you're licensed; add Medicaid through the CLTC waivers (fee-for-service). SC personal care is billed hourly.

Your four revenue streams:

Source	What it is	Key fact
Private pay	Client/family pays out of pocket, hourly	Start immediately; ~\$30/hr in SC (2024, below national)
Medicaid — CLTC waivers	Community Choices & other HCBS waivers	Personal/attendant care; fee-for-service (not managed care); requires enrollment & EVV
Long-term-care insurance	The client's LTC policy reimburses in-home care	Bill the family or insurer once benefit triggers are met
Veterans (VA)	VA Homemaker/HHA, Veteran-Directed Care, Aid & Attendance	Serve veterans via the Community Care Network (Region 3, Optum)

8.1 Private pay — your launch market

The day your license issues, serve private-pay clients — families paying out of pocket, typically billed hourly with a visit minimum. South Carolina is a lower-cost market: non-medical homemaker care runs about \$30/hour (2024 cost-of-care data, roughly 14% below the national median — verify the current figure). Set your rate above your fully-loaded caregiver cost. Long-term-care insurance is part of this stream: many policies reimburse in-home personal care once ADL/cognitive triggers are met (some require a licensed provider) — you bill the family or the insurer.

8.2 Medicaid — the CLTC waivers

SC Medicaid's Division of Long Term Living (CLTC) runs the HCBS waivers that pay for personal and attendant care. The main one for a non-medical agency is the Community Choices Waiver (covering Personal Care I/II and Attendant Care, plus companion care and respite). A key South Carolina fact: after the January 1, 2026 managed-care carve-in, waiver personal care stays FEE-FOR-SERVICE through SCDHHS/CLTC — you bill the state, not an MCO.

- 1. Hold your DPH license first (Part 4).**
- 2. Enroll as an SC Medicaid provider.** Use the online Provider Enrollment Web Application (providerservices.scdhhs.gov); Provider Service Center (888) 289-0709. You have 30 days to complete a started application.
- 3. Take CLTC referrals.** Community Choices members are authorized and referred through CLTC case management.
- 4. Set up AuthentiCare EVV (Part 6)** before you deliver a Medicaid visit.

8.3 The rate & billing

On the SC HCBS Waiver Fee Schedule (updated November 1, 2025), Personal Care II (T1019) pays \$25.00/hour; Personal Care I (S5130) is around \$22/hour and Attendant Care (S5125) around \$16.20/hour — verify the current figures against the live schedule. Bill SCDHHS fee-for-service against verified AuthentiCare visits, and confirm the timely-filing window.

8.4 Veterans

- **VA Homemaker / Home Health Aide & Veteran-Directed Care:** the VA's in-home programs — contract through the local VA medical center (Dorn VAMC in Columbia, 803-776-4000; Ralph H. Johnson VAMC in Charleston, 843-577-5011) and the VA Community Care Network. South Carolina is in Region 3, administered by Optum.
- **Aid & Attendance:** an increased VA pension (2025–2026: up to about \$2,424/month for a single veteran — verify on [VA.gov](https://va.gov)) applied toward your private-pay care.

PART 9

What It Costs & How Long It Takes

In short: Budget the license, insurance & bonding, caregiver screening/training, EVV, payroll float, and (optionally) accreditation. Launch on private pay, then add Medicaid.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
DPH license	Initial IHCP license	\$1,000 (renewal \$800)
Business setup	SOS LLC \$110 (no annual report), EIN free, NPI free, local business license, legal	~\$110 + local + legal
Insurance & bond	Liability + abuse + non-owned auto + dishonesty bond + workers' comp	Get quotes — annual
Caregiver screening/training	SLED check + drug test + TB + your training program	Per caregiver
EVV & software	AuthentiCare + scheduling/billing/payroll platform	Monthly SaaS
Payroll float	You pay caregivers before Medicaid/insurers pay you	Working capital
Policies	P&P Manual + Forms Packet	One-time
Accreditation (optional)	ACHC / CHAP / Joint Commission	~\$7,000–\$14,500 / 3-yr cycle

The cash-flow trap to plan for. You pay caregivers weekly, but Medicaid and insurers pay weeks later. Private-pay clients smooth this out — line up a business line of credit before you scale Medicaid hours.

9.2 Timeline

Weeks	Entity & policies	Form the LLC, write your P&P to Reg 61-122, get local license
Weeks	Insurance	Bind coverage & bond
Submit	DPH application	Application + \$1,000 to Healthcare Facility Licensing
Right away	Start private pay	Serve private-pay & LTC-insurance clients
Parallel	Enroll for Medicaid	SCDHHS enrollment + AuthentiCare EVV
Ongoing	Referrals & census	Discharge planners, AAAs, CLTC case managers

9.3 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$30/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

9.4 The hidden costs new owners miss

The line items above are the visible costs. The ones that sink underprepared owners are the invisible ones — plan for these before you open:

- **Payroll float:** you pay caregivers every week or two, but Medicaid (SCDHHS/CLTC) and long-term-care insurers pay you weeks after you bill. On a growing Medicaid caseload you can be out tens of thousands of dollars in wages you've paid but not yet collected. Keep several weeks of payroll in reserve, or lean on private pay (which pays up front) while your Medicaid line ramps.
- **Unbillable overhead:** your own time, a scheduler/coordinator, office and phone, background checks and drug screens, and caregiver training — none of it is billable, all of it is real.
- **Turnover churn:** every caregiver who quits costs you re-recruiting, re-screening, and re-training. Budget for it, and spend on retention instead — it is far cheaper than replacement.
- **Denied and delayed claims:** a Medicaid claim that doesn't match a verified AuthentiCare (EVV) visit, or that misses the timely-filing window, simply doesn't get paid. Reconcile every remittance and chase exceptions weekly — unreconciled claims are money you earned and lost.

PART 10

Getting Your First Clients

In short: Clients come from discharge planners, home-health/hospice partners, the 10 Area Agencies on Aging, CLTC case managers, care managers, and senior communities. Build these before you open.

10.1 The referral sources

- **Hospital discharge planners & case managers:** the highest-volume source for post-hospital home support — make same-day starts easy.
- **Home-health & hospice partners:** they refer the non-skilled hours their benefit doesn't cover; build reciprocal partnerships.
- **Area Agencies on Aging & the SC Department on Aging:** South Carolina has 10 AAAs (on the Council of Governments regions), coordinated by the SC Department on Aging — Aging Helpline 1-800-868-9095 — and the GetCareSC portal. Get listed and known.
- **CLTC case managers:** for waiver clients, CLTC case management authorizes services and refers to enrolled providers.
- **Geriatric care managers & senior communities:** Aging Life Care professionals and assisted-/independent-living communities refer supplemental one-on-one care.
- **SC 2-1-1:** list your services with SC 2-1-1 (dial 211) — a free statewide referral channel.

10.2 What to ask each referral source

“How do you decide which home-care agency to refer a family to?”

“What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?”

“Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?”

“How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?”

“What makes an agency easy to work with, and what gets one dropped from your list?”

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the administrator & staff

Who	Contact	What to verify / ask
SC DPH — Healthcare Facility Licensing	(803) 545-4215 · dph.sc.gov (In-Home Care Providers)	The IHCP application & \$1,000 fee; the current intake contact; inspections
SLED — background checks	sled.sc.gov	The criminal-background check; whether fingerprint/FBI is required
City/County business licensing	your jurisdiction	Your local business license (no statewide license in SC)
Caregiver training resources	your own program / vendors	Reg 61-122 §300.D training topics (no state minimum hours)

Business, insurance & billing IDs

Who	Contact	What to verify / ask
SC Secretary of State	sos.sc.gov · businessfilings.sc.gov	Forming your LLC (\$110); no LLC annual report
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPPEs / CMS	npes.cms.hhs.gov	Your Type 2 NPI (for Medicaid/VA billing)
Insurance agent (home-care program)	trustedchoice.com ; carriers in Part 3.3	Liability + abuse + non-owned auto + dishonesty bond + workers' comp

Funding, EVV & referrals

Who	Contact	What to verify / ask
SCDHHS — provider enrollment	(888) 289-0709 · providerservices.scdhhs.gov	Enrolling your NPI; taking CLTC referrals
SCDHHS / CLTC (Division of Long Term Living)	scdhhs.gov (waivers)	The Community Choices waiver; the current PC/attendant rates
SC Medicaid — EVV / AuthentiCare	scdhhs.gov (EVV)	AuthentiCare onboarding and compliance
VA Community Care / Optum (Region 3)	va.gov/communitycare	The VA Homemaker/HHA program; joining the CCN

Referral & senior-services partners

Who	Contact	Role
SC Department on Aging / AAAs (10)	1-800-868-9095 · aging.sc.gov · getcaresc.com	Local information & referral for older adults
SC 2-1-1	dial 211 · sc211.org	Free statewide referral line — list your services
Accreditor (optional) — ACHC/CHAP/TJC	achc.org (Cary, NC) · chapinc.org · jointcommission.org	Whether/when to accredit; payer requirements
Hospital / SNF discharge planners	your local hospitals & SNFs	The highest-volume referral source

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I really need the DPH license — can't I just run companion/sitter care?

Only if your service is truly pure companionship with zero help with daily living. South Carolina does NOT broadly exempt companion/sitter businesses the way Florida does, so any personal care — bathing, dressing, transferring — puts you under the In-Home Care Provider license. When in doubt, confirm your status with DPH before you advertise or take a client.

Do my caregivers need to be certified nurse aides?

No. IHCP caregivers are not certified nurse aides, and Reg 61-122 sets no minimum training hours and no RN-supervision mandate. That light-touch standard is exactly why your own screening and training program becomes your competitive edge (Part 5).

You can do this

Opening a South Carolina in-home care agency is very doable — a single license, light staffing rules, and a real fee-for-service Medicaid market alongside private pay. Take it one rung at a time: form the business, write your policies to Reg 61-122, get the DPH license, recruit and screen great caregivers, launch on private pay, add Medicaid through SCDHHS/CLTC, and build referral relationships. Verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in SC Code Title 44, Chapter 70, Regulation 61-122, and current SC Medicaid information as of 2026. This is general information, not legal, tax, or billing advice.

Requirements, fees, rates, and contacts change — verify each with DPH, SCDHHS, and qualified counsel before you rely on it. Confirm the current license fee, the CLTC personal-care rates, and your EVV setup before you rely on them.